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REPORT

PT Name	: **BASAMMA KUNCHIKORVE**	**Age :** 60 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. VIJAY PANIKAR**	**Registered on** : 31-08-2026 12:38 PM	
Reg No	: **HL3701831939 / MH035**	**Received on** : 31-08-2026 03:22 PM	
Barcode	: **HEN120873**	**Reported on** : 31-08-2026 04:02 PM	
INV	: ALBUMIN/CREATININE RATIO (URINE)		**SAMPLE :** Urine

ALBUMIN/CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	21.64	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	164.60	mg/dL	28.0 - 217.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	13.15	mg/g	0 - 30

Clinical Information :

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with diabetes, appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as high blood pressure, which can sometimes lead to kidney disease.

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **VALU JEUGHALE**
Ref By : **DR. SELF**
Reg No : **HL3801822048 / MH118/H5776**
Barcode : **FZ648520**
INV : ALBUMIN/CREATININE RATIO (URINE), 24 Hours

Age : 60 Year | **Sex :** Male
Registered on : 21-08-2026 08:26 PM
Received on : 22-08-2026 03:36 AM
Reported on : 22-08-2026 11:37 AM

SAMPLE COLLECTED AT :
SHREE RAM CLINICAL LAB

SAMPLE : Urine

ALBUMIN/CREATININE RATIO (URINE), 24 HOURS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Total volume	1000	ML/day	250 - 2400
MICROALBUMIN Method: PHOTOMETRIC	**450.0**	mg/dl	0 - 30
Urine Creatinine Method: Enzymatic	81.5	mg/dL	28 - 217
Creatinine 24 Hours (Urine) Method: Spectrophotometry	815	mg/dL	600 - 2400
Microalbumin 24 Hours (Urine) Method: Spectrophotometry	4500	mg/dL 24 Hrs	. - .
Microalbumin / Creatinine Ratio Method: CALCULATED	5.52	mg/dL	0 - 30

Interpretation :

- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

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Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **SNEHAL SUPNWAR**
Ref By : **DR. VISHNUDAS KHANDADE**
Reg No : **HL8501827691 / MH451**
Barcode : **HEL148676**
INV : Anti-NMO (AQUAPORIN 4)

Age : 21 Year | **Sex :** Female
Registered on : 26-08-2026 06:53 PM
Received on : 27-08-2026 10:17 AM
Reported on : 29-08-2026 05:08 PM

SAMPLE COLLECTED AT :
SHRI CLINICAL LABORATORY, BARURE HOSPITAL LATUR

SAMPLE : Serum

ANTI-NMO (AQUAPORIN 4)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
AQP4 (NMO-IgG) Autoantibody Test Method: ELISA	1.4	u/mL	0.0 - 3.0

DIAGNOSIS OF :
* Diagnosis of neuromyelitis optica spectrum disorder (NMOSD).
* Diagnosis of autoimmune AQP4 channelopathy.
* Diagnosis of neuromyelitis optica (NMO).
* Distinguishing NMOSD from multiple sclerosis early in the course of disease.

CLINICAL INFORMATION :
Neuromyelitis optica (NMO), sometimes called Devic disease or opticospinal multiple sclerosis (MS) is a severe, relapsing, autoimmune, inflammatory and demyelinating central nervous system disease that predominantly affects optic nerves and spinal cord.
Detection of NMO/APQ4-IgG allows distinction of NMOSD from MS and is indicative of a relapsing disease, mandating initiation of immunosuppression, even after the first attack, thereby reducing attack frequency and disability in the future.

INTERPRETATION :
A positive value is consistent with a neuromyelitis optica spectrum disorder (NMOSD) and justifies initiation of appropriate immunosuppressive therapy at the earliest possible time. This allows early initiation and maintenance of optimal therapy. Recommend follow-up in 3 to 6 months if NMOSD is suspected.

CAUTIONS :
AQP4-IgG antibodies may drop below detectable levels in setting of therapies for acute attack (IV methylprednisolone or plasmapheresis) or attack prevention (immunosuppressants).

~~End of report~~

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Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MRS FARHA QURESHI**
Ref By : **DR. KHATOON BI KAZI HOSPITAL**
Reg No : **HL3001829555 / RAJ14**
Barcode : **HEL095627**
INV : CORTISOL EVENING (RANDOM)

Age : 38 Year | **Sex :** Female
Registered on : 28-08-2026 06:45 PM
Received on : 29-08-2026 02:04 AM
Reported on : 29-08-2026 07:01 AM

SAMPLE COLLECTED AT :
AZIZ CLINICAL LAB

SAMPLE : Serum

CORTISOL EVENING (RANDOM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
CORTISOL EVENING Method: CLIA	**1.50**	ug/dL	3.44 - 16.7

.

Cortisol, the main glucocorticoid (representing 75%-90% of the plasma corticoids) plays a central role in glucose metabolism and in the body's response to stress.

Cortisol levels are regulated by adrenocorticotropic hormone (ACTH), which is synthesized by the pituitary gland in response to corticotropin-releasing hormone (CRH). CRH is released in a cyclic fashion by the hypothalamus, resulting in diurnal peaks (6 a.m.-8 a.m.) and troughs (11 p.m.) in plasma ACTH and cortisol levels.

The majority of cortisol circulates bound to cortisol-binding globulin (CBG-transcortin) and albumin. Normally, less than 5% of circulating cortisol is free (unbound). The free cortisol is the physiologically active form and is filterable by the renal glomerulus.

Although hypercortisolism is uncommon, the signs and symptoms are common (eg, obesity, high blood pressure, increased blood glucose concentration). The most common cause of increased plasma cortisol levels in women is a high circulating concentration of estrogen (eg, estrogen therapy, pregnancy) resulting in increased concentration of cortisol-binding globulin.

Spontaneous Cushing syndrome results from overproduction of glucocorticoids as a result of either primary adrenal disease (adenoma, carcinoma, or nodular hyperplasia) or an excess of ACTH (from a pituitary tumor or an ectopic source). ACTH-dependent Cushing syndrome due to a pituitary corticotroph adenoma is the most frequently diagnosed subtype; most commonly seen in women in the third through fifth decades of life. The onset is insidious and usually occurs 2 to 5 years before a clinical diagnosis is made.
Causes of hypocortisolism are:
-Addison disease-primary adrenal insufficiency
-Secondary adrenal insufficiency:
--Pituitary insufficiency
--Hypothalamic insufficiency
-Congenital adrenal hyperplasia-defects in enzymes involved in cortisol synthesis

~~End of report~~

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Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **SWATI SURYAWANSHI**	**Age :** 40 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. PIYUSH BANSAL MBBS , MS**	**Registered on :** 29-08-2026 03:24 PM	
Reg No	: **HL3401830348 / MH160**	**Received on** : 30-08-2026 08:56 AM	
Barcode	: **HEL148947**	**Reported on** : 02-09-2026 10:51 AM	
INV	: FLUORESCENT TREPONEMAL ANTIBODIES; FTA-ABS, SERUM		**SAMPLE :** Serum

FLUORESCENT TREPONEMAL ANTIBODIES; FTA-ABS, SERUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
FLUORESCENT TREPONEMAL ANTIBODIES; FTA-ABS, Method: Indirect Immunofluorescence	NEGATIVE	.	. - NEGATIVE

Note :

1. A negative result does not exclude the possibility of exposure to infection by Treponema pallidum.
2. False reactive reactions in FTA-ABS are seen in 1% population in patients suffering from chronic diseases like Leprosy & Malaria and Autoimmune diseases like SLE
3. This test is used for confirmation of syphilis due to its high sensitivity and specificity .

Comments :

Syphilis is a systemic infectious disease caused by Treponema pallidum. Early Syphilis is not life threatening, but late manifestations affect the life span and patient's productivity. Untreated primary infections in the mother affect the fetus.

STATE OF THE DISEASE	PERCENTAGE POSITIVITY
Primary Syphilis	80-90
Secondary Syphilis	99-100
Tertiary Syphilis	98

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **ARPIT SHRIMAL (505615)**	**Age :** 36 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. PR HOSPITAL**	**Registered on :** 31-08-2026 04:47 PM	
Reg No	: **HL8101832417 / MH300**	**Received on** : 01-09-2026 07:59 AM	
Barcode	: **HEJ072991**	**Reported on** : 02-09-2026 04:14 PM	**SAMPLE :** Urine
INV	: PHENOL URINE, RANDOM		

PHENOL URINE, RANDOM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PHENOL URINE, RANDOM Method: HPLC	6.8	mg/L	0.0 - 10.0

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name	: **SURJYA UCHOI**	**Age :** 44 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** DNA LAB
Ref By	: **DR. TATA HOSPITAL**	**Registered on :** 26-08-2026 12:06 PM	
Reg No	: **HL2901826963 / MH081**	**Received on** : 26-08-2026 04:59 PM	
Barcode	: **HEN102473**	**Reported on** : 28-08-2026 03:19 PM	**SAMPLE :** EDTA_PLASMA
INV	: Plasma Metanephrine		

PLASMA METANEPHRINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Metanephrine- Method: ELISA	35	pg/ml	0.0 - 65.0

Interpretation :

The assay detects Plasma free metanephrines which are metabolites of epinephrine. Epinephrine is catecholamine hormones important in blood pressure regulation & produced in the adrenal medulla. It is released in small fluctuating quantities in both the blood and urine. Increased levels can be seen in phaechromocytoma. Falsely elevated levels may be seen with ingestion of coffee, tobacco, tea, alcohol, drugs like acetoaminophen.

~~End of report~~

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Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **SHAILESH SINGH**	**Age :** 40 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. UMESH KHANNA**	**Registered on :** 01-09-2026 02:11 PM	MILLENNIUM PATHOLOGY LAB
Reg No	: **HL4701832597 / HYMH/NALSPR/MILL1**	**Received on** : 01-09-2026 05:49 PM	
Barcode	: **HED028108**	**Reported on** : 01-09-2026 06:17 PM	
INV	: SPOT Urine Protein/Creatinine Ratio		**SAMPLE :** Urine

SPOT URINE PROTEIN/CREATININE RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Urinary Protein, Total Method: Spectrophotometry	15.61	mg/dL	0.0 - 25.0
Urinary Creatinine Method: Spectrophotometry	84.68	mg/dL	30.0 - 250.0
PROTEIN/CREATNINE RATIO Method: CALCULATED	0.18	RATIO	0 - 0.20

Interpretation :

PROTEIN CREATININE RATIO	REMARKS
<0.20	Normal
0.20-1.00	Low grade proteinuria
1.00-5.00	Moderate proteinuria
>5.00	Nephrosis

Comments :

Urinary total proteins are nearly negligible in healthy adults. The Protein Creatinine ratio is a simple and convenient method to quantitate and monitor proteinuria in adults with chronic kidney disease. Patients with 2 or more positive results within a period of 1-2 weeks should be labeled as having persistent proteinuria and investigated further.

~~End of report~~

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Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **HARISHCHANDRA SAMBARE**	**Age :** 70 Y | **Sex :** Male	**SAMPLE COLLECTED AT :** JIJAU SANSTHA RATNAGIRI
Ref By	: **DR. JIJAU HOSPITAL, RATNAGIRI**	**Registered on :** 29-08-2026 09:22 PM	
Reg No	: **HL2101830651 / MH004/012/DEV55**	**Received on** : 30-08-2026 08:53 AM	
Barcode	: **HEE176953**	**Reported on** : 02-09-2026 10:51 AM	
INV	: SWAB C/S		**SAMPLE :** SWAB

SWAB C/S

TEST DESCRIPTION	RESULT
Results Method: Disc Diffusion	No growth
Incubation period	obtained after 24 / 48 hrs of incubation at 37°c
Colony count	.

REMARKS :
Condition which can cause a false negative Cultures :

1. Patient is on Antibiotics.
2. Anaerobic bacteria.
3. Fastidious aerobic bacteria which are not able to grow on routine culture media.
4. Besides all these factors, at least in 25-40 % of cases there is on direct correlation between in vivo clinincal pictures and in-vitro culture result.

Interpretation :
(++++) or (+++) : maximum sensitivity
(++) : moderate sensitivity
(+) : minimum sensitivity
(R) : resistant

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. AMIT AGRAWAL (MD MICROBIOLOGY)
Consultant Microbiology

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MR. INDRA KUMAR HARI LAL PATWA**
Ref By : **DR. SHUBHASH GUPTA**
Reg No : **HL3301827319 / RAJ44**
Barcode : **HED056736**
INV : TACROLIMUS

Age : 31 Year | **Sex :** Male
Registered on : 26-08-2026 02:54 PM
Received on : 26-08-2026 06:20 PM
Reported on : 28-08-2026 07:55 AM

SAMPLE COLLECTED AT :
MATOSHREE LAB

SAMPLE : EDTA Blood

TACROLIMUS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Tacrolimus	4.35	ng/mL	. - .
Method: chemiluminescent microparticle immunoassay			

INTERPRETATION :

KIDNEY TRANSPLANT RECIPIENT	RANGES FOR TROUGH LEVEL (ng/ml)
1 Month post-transplant	12-15
2 to 3 months post-transplant	10 to 12
4 to 6 months post-transplant	05 to 10
>6 months in selected patients levels	<5, may be sufficient if graft function is stable but immunosuppression is highly variable
IN LIVER TRANSPLANT RECIPIENTS	05 to 20
HEART TRANSPLANT RECIPIENTS	
PEDIATRIC	
0-12 months post-transplant	10-15
>12 months post-transplant	5-10
ADULT	
0-3 months post-transplant	10-15
3-6 months post-transplant	8-12
6-12 months post-transplant	6-12
>12 months post-transplant	6-10

TACROLIMUS RESULTS CAN SHOW VARIATIONS DEPENDING ON FOLLOWING REASONS -
* DIFFERENT ROUTE OF ADMINISTRATION, TIME OF COLLECTION
* TIME AFTER TRANSPLANT, TYPE OF ALLOGRAFT
* DIFFERENT AMOUNT & TIME OF DOSAGE, DRUG DISTRIBUTION WHICH IN TURN DEPENDS ON HEMATOCRIT & PLASMA PROTEINS CONCENTRATION.
* DIFFERENT DRUG COMPANY
* OTHER CO-MORBID MEDICAL CONDITIONS, CONCOMITANT USE OF IMMUNOSUPPRESSION & OTHER DRUG THAT INHIBIT (CALCIUM CHANNEL BLOCKERS, ANTIFUNGAL AGENTS, SOME ANTIBIOTICS) OR INDUCE (ANTICONVULSANTS, RIFAMPIN) METABOLISM FOR TACROLIMUS.
* DIFFERENT TEST METHODOLOGIES CAN YIELD DIFFERENT RESULTS, IT IS ADVISABLE TO MONITOR THE DRUG LEVELS USING THE SAME TESTING METHOD & LABORATORY.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **POOJA MOHITE**	**Age :** 35 Y | **Sex :** Female	**SAMPLE COLLECTED AT :** 6387305523
Ref By	: **DR. DR. RAKESH PANDEY**	**Registered on :** 26-08-2026 05:06 PM	
Reg No	: **HL9901827572 / MH457**	**Received on** : 26-08-2026 06:55 PM	
Barcode	: **hen122821**	**Reported on** : 30-08-2026 04:09 PM	**SAMPLE :** Tissue
INV	: TISSUE C/S Anaerobic		

TISSUE C/S ANAEROBIC

TEST DESCRIPTION	RESULT
Results Method: Disc Diffusion	No growth
Incubation period	obtained after 24 / 48 hrs of incubation at 37°c
Colony count	.

REMARKS :
Condition which can cause a false negative Cultures :

1. Patient is on Antibiotics.
2. Anaerobic bacteria.
3. Fastidious aerobic bacteria which are not able to grow on routine culture media.
4. Besides all these factors, at least in 25-40 % of cases there is on direct correlation between in vivo clinincal pictures and in-vitro culture result.

Interpretation :
(++++) or (+++) : maximum sensitivity
(++) : moderate sensitivity
(+) : minimum sensitivity
(R) : resistant

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Consultant Pathologist

Dr. AMIT AGRAWAL (MD MICROBIOLOGY)
Consultant Microbiology

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **MR. VAIBHAV SAWANT**	**Age :** 35 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. DR. ANAND RAM**	**Registered on :** 28-08-2026 04:53 PM	
Reg No	: **HL5901829518 / HYMH/KDV/H1886**	**Received on** : 28-08-2026 05:28 PM	
Barcode	: **HEN173395**	**Reported on** : 28-08-2026 06:32 PM	
INV	: Troponin - I (Quantitative)		**SAMPLE :** Serum

TROPONIN - I (QUANTITATIVE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TROPONIN I, SERUM	7.94	pg/mL	0.0 - 25.0
Method: HIGH SENSITIVE (CMIA)			

COMMENTS :

Troponin-I is a cardiac marker elevated only in patients suffering from acute Myocardial Infarction. Patients with renal disease or acute muscle injury show normal levels. High sensitivity assays can detect elevated levels of Troponin-I (above the 99th percentile of an apparently healthy reference population) within 3 hours after the onset of chest pain.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)